SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Pembrolizumab 400 mg · colorectal

First-line metastatic MSI-H / dMMR colorectal cancer · NCCP 00558 · version 13 · 42-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
Pembrolizumab aims to control bowel cancer that has spread and has a particular DNA-repair feature, called MSI-H or dMMR. Your team discusses likely benefit, uncertainty and alternatives for you.

Schedule
Pembrolizumab 400 mg is given over about 30 minutes on Day 1 every 6 weeks (42 days). The visit takes longer for assessment and preparation.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Pembrolizumab
IMMUNOTHERAPY

- ☐ Immune inflammation of lungs, bowel, liver, kidneys, hormone glands, skin, heart and nervous system
- ☐ Infusion reactions
- ☐ Some effects need steroids, hospital care or long-term hormone replacement
- ☐ Effects may start after the final dose

Discuss the first-line MSI-H/dMMR colorectal indication, benefit and uncertainty, alternatives, immune inflammation in multiple organs, possible severe or lasting effects, infusion reactions, steroids or long-term hormone replacement, pregnancy precautions and the option to reconsider. Confirm the urgent contact plan.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00558 v13.

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